

Policy on Community Emergencies, Epidemics and Other Disasters

«Hospital Name»

Document control

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Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

The organisation shall identify potential community emergencies, epidemics and other disasters and manage them through a documented plan. Resources shall be available according to threat perception and expected workload. The plan shall be tested at least twice a year, with at least one mock drill once in 12 months.

1. Purpose

This policy establishes how the organisation identifies and manages community emergencies, epidemics and other disasters that may cause a sudden rush of victims.

2. Scope

This policy applies to the organisation's planning and preparedness for community emergencies, epidemics and other disasters relevant to its geographical location and the community served.

3. Policy standards

3.1 Identification of potential emergencies

The organisation shall identify potential community emergencies, epidemics and other disasters likely to cause a sudden rush of victims.

The potential emergencies shall be identified based on geographical location and the community served by the organisation.

Examples include earthquake, floods, train accident, civil unrest outside the organisation's premises, major fire and outbreak of disease/epidemics. These are examples, not a fixed list.

For example, an organisation in an industrial town shall identify the industrial hazards that may occur in its vicinity.

3.2 Documented disaster, community emergency and epidemic plan

The disaster, community emergency and epidemic plan must incorporate:

- alert code
- information and communication
- action cards for each of the staff
- availability and earmarking of resources including adequacy of medical supplies, equipment, materials and trained personnel
- establishment of command nucleus
- training and mock drills
- managing clinical activities during the event

The plan shall also include:

- activating and deactivating the plan
- receiving, identifying and triaging casualties
- defined areas for reception and treatment for casualties
- transportation aids
- communication aids
- managing visitors and controlling the movement of individuals and vehicles
- relocating/discharging admitted patients wherever needed

The plans shall conform to the relevant local laws and national plans on disaster management.

The emergency room could follow triage policy according to the National Disaster Management Authority (NDMA) guidelines. A good reference is NDMA guidelines.

3.3 Medical supplies, equipment and materials

Resource availability shall be according to threat perception.

The number of resources, including medical consumables and equipment, shall be commensurate with the expected workload.

3.4 Testing of the plan

Testing twice a year is only the minimum frequency, and this may be increased.

In case the organisation has different plans for different disasters, each of the plans shall be tested at least twice a year.

The plan can be tested using a table-top exercise, or a mock drill.

At a minimum, at least one mock drill shall be held once in 12 months.

This shall test all the components of the plan and not just awareness.

In the case of a mock drill, simulated patients, not real patients, shall be used.

After every table-top exercise/mock drill, the variations are identified, the reason for the same is analysed, debriefing conducted and where appropriate the necessary corrective and/or preventive actions are taken.

4. Non-negotiable rules

- Do not leave potential community emergencies, epidemics or other disasters unidentified when they are relevant to the organisation's geographical location and the community served.
- Do not manage a community emergency, epidemic or other disaster without the documented plan incorporating the required alert code, information and communication, action cards, resources, command nucleus, training and mock drills, and management of clinical activities during the event.
- Do not use a documented disaster, community emergency and epidemic plan that omits activation and deactivation, casualty reception/identification/triage, defined reception and treatment areas, transportation aids, communication aids, visitor and movement management, and relocation/discharge of admitted patients wherever needed.
- Do not use the plan without conformity to relevant local laws and national plans on disaster management.
- Do not provide medical supplies, equipment and materials at levels that are not according to threat perception and commensurate with expected workload.
- Do not treat twice-yearly plan testing as optional.
- Where different plans exist for different disasters, do not leave any of those plans untested at least twice a year.
- Do not count testing that covers only awareness as sufficient; testing shall cover all components of the plan.
- Do not use real patients in a mock drill; simulated patients shall be used.
- Do not close a table-top exercise or mock drill without identifying variations, analysing the reason for the variations, conducting debriefing and, where appropriate, taking the necessary corrective and/or preventive actions.

5. What we do

5.1 Identify relevant emergencies

Identify potential community emergencies, epidemics and other disasters likely to cause a sudden rush of victims.

Base identification on geographical location and the community served.

Identify industrial hazards that may occur in the vicinity when relevant to an organisation in an industrial town.

5.2 Maintain the documented plan

Maintain the alert code, information and communication arrangements, action cards for each staff member, resource availability and earmarking, command nucleus, training and mock drills, and arrangements for managing clinical activities during the event.

Include activation and deactivation, casualty reception/identification/triage, defined casualty reception and treatment areas, transportation aids, communication aids, visitor and movement management, and relocation/discharge of admitted patients wherever needed.

Ensure conformity with relevant local laws and national plans on disaster management.

The emergency room could follow triage policy according to NDMA guidelines.

5.3 Maintain resources

Make resources available according to threat perception.

Ensure the number of medical consumables, equipment and other resources is commensurate with expected workload.

5.4 Test and improve the plan

Test the plan at least twice a year.

Where different plans exist for different disasters, test each plan at least twice a year.

A table-top exercise or mock drill can be used for testing.

Hold at least one mock drill once in 12 months.

Test all components of the plan and not just awareness.

Use simulated patients, not real patients, for mock drills.

After every table-top exercise/mock drill, identify variations, analyse the reason, conduct debriefing and, where appropriate, take corrective and/or preventive actions.

6. Governance and responsibility

Role	Responsibility
Organisation	The organisation is responsible for identifying relevant community emergencies, epidemics and other disasters, maintaining the documented plan, making required resources available and ensuring the plan is tested.
All staff	The plan includes an established command nucleus and action cards for each staff member, defining individual responsibilities during the event.
Emergency Room	The emergency room could follow triage policy according to NDMA guidelines where the organisation chooses to adopt it.

7. Quality monitoring

Monitoring area	What is monitored
Plan testing frequency	Completion of plan testing at least twice a year
Disaster-specific plan testing	Testing of each disaster-specific plan at least twice a year where different plans exist
Mock drill frequency	Completion of at least one mock drill once in 12 months
Testing coverage	Coverage of all plan components during testing
Mock-drill patients	Use of simulated patients rather than real patients in mock drills
Post-exercise actions	Identification of variations, analysis of reasons, debriefing and corrective/preventive action where appropriate after every exercise

8. Training and staff acknowledgement

Training and mock drills are incorporated into the documented disaster, community emergency and epidemic plan.

I have read the Policy on Community Emergencies, Epidemics and Other Disasters of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

The documented disaster, community emergency and epidemic plan, including action cards for each staff member, shall be available to the personnel involved in managing the event.

10. Abbreviations

Abbreviation	Meaning
NDMA	National Disaster Management Authority

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.4.a	Commitment*	Sections 3.1 and 5.1 address identification of potential community emergencies, epidemics and other disasters based on geographical location and the community served.
COP.4.b	Commitment*	Sections 3.2 and 5.2 address the documented disaster, community emergency and epidemic plan with all 14 required plan elements and conformity to relevant laws and national plans.
COP.4.c	Commitment	Sections 3.3 and 5.3 address availability of medical supplies, equipment and materials according to threat perception and commensurate with expected workload.
COP.4.d	Commitment	Sections 3.4 and 5.4 address testing at least twice a year, at least one mock drill once in 12 months, all-component coverage, simulated patients and four mandatory post-exercise steps.

12. Required Records/Evidence Checklist

Emergency identification

- Documented list of potential community emergencies, epidemics and other disasters identified for the organisation's geographical location and community.
- Identification of relevant hazards in the vicinity, including industrial hazards where relevant.

Documented disaster, community emergency and epidemic plan

- Current documented plan containing the alert code; information and communication; action cards for each staff member; availability and earmarking of medical supplies, equipment, materials and trained personnel; command nucleus; training and mock drills; and management of clinical activities during the event.
- Plan covering activation and deactivation; casualty reception, identification and triage; defined casualty reception and treatment areas; transportation aids; communication aids; visitor management and control of individual and vehicle movement; and relocation/discharge of admitted patients wherever needed.
- Plan showing conformity with relevant local laws and national plans on disaster management.
- NDMA guidelines retained as a reference; any emergency-room triage policy based on NDMA guidelines where the organisation chooses to follow it.

Resources

- Arrangements showing availability and earmarking of medical supplies, equipment, materials and trained personnel according to threat perception and expected workload.

Testing and exercises

- Testing records showing the plan was tested at least twice a year.
- Where different disaster plans exist, testing records for each plan showing testing at least twice a year.
- Record of at least one mock drill once in 12 months.
- Exercise records showing all components of the plan were tested, not just awareness.
- Mock-drill records showing simulated patients, not real patients, were used.
- After-exercise records showing variations identified, reasons analysed, debriefing conducted and corrective and/or preventive actions taken where appropriate.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.4.
2. National Disaster Management Authority (NDMA) guidelines.
3. Relevant local laws and national plans on disaster management.

Disclaimer

This policy reorganises the supplied COP.4 objective elements and Guidebook interpretation into plain language. Mandatory requirements and their stated modal strength have been retained. Illustrative examples, optional approaches and references have not been converted into mandatory requirements.